

QUEENSLAND INDUSTRIAL RELATIONS COMMISSION

CITATION: *Richards v Workers' Compensation Regulator*
[2024] QIRC 241

PARTIES: **Richards, Peter**
(Appellant)

v

Workers' Compensation Regulator
(Respondent)

CASE NO: WC/2021/18

PROCEEDING: Appeal against decision of Workers'
Compensation Regulator

DELIVERED ON: 4 October 2024

HEARING DATES: 7 September 2022, 8 September 2022,
9 September 2022, and 25 November 2022

SUBMISSIONS: Appellant's closing submissions: 10 January
2023
Respondent's closing submissions: 15 February
2023
Appellant's closing submissions in reply:
23 February 2023

MEMBER: Power IC

HEARD AT: Brisbane

ORDERS:

1. **The appeal is dismissed.**
2. **The decision of the Workers' Compensation Regulator is confirmed.**
3. **Failing agreement on costs, to be the subject of a further application to the Commission.**
4. **Liberty to Apply.**

CATCHWORDS: WORKERS' COMPENSATION –

ENTITLEMENT TO COMPENSATION – whether injury was suffered pursuant to s 32 of the *Workers' Compensation Act 2003* (Qld) – whether appellant suffered an aggravation of his personal injury which arose out of, or in the course of, his employment – whether employment was a significant contributing factor – appellant did not suffer an aggravation of his personal injury which arose out of, or in the course of, his employment – review decision of respondent confirmed

LEGISLATION:

Workers' Compensation and Rehabilitation Act 2003 (Qld), s 32

CASES:

Briffa v Q-COMP (2005) 180 QGIG 70
McDonald v Q-COMP (No. 2) (2008) 188 QGIG 180
Qantas Airways Limited v Q-COMP (2006) 181 QGIG 301
Rossmuller v Q-COMP [2010] ICQ 4
WorkCover Queensland v BHP (Qld) Workers Compensation Unit (2002) 170 QGIG 142

APPEARANCES:

Mr D.V. Nguyen of Counsel instructed by Black and Co Lawyers

Ms M.J. Brooks of Counsel directly instructed by the Respondent

Reasons for Decision

- [1] Mr Peter Richards ('the Appellant') appeals a decision of the Workers' Compensation Regulator ('the Respondent') to reject his application for compensation relating to an injury to his left shoulder pursuant to s 32 of the *Workers' Compensation and Rehabilitation Act 2003* (Qld) ('the Act').
- [2] The issue to be determined is whether the Appellant suffered a personal injury within the meaning of s 32 of the Act. Section 32 of the Act provides:

- (1) An *injury* is personal injury arising out of, or in the course of, employment if the employment is a significant contributing factor to the injury.

...

- (3) *Injury* includes the following –

- (a) a disease contracted in the course of employment, whether at or away from the place of employment, if the employment is a significant contributing factor to the disease;
- (b) an aggravation of the following, if the aggravation arises out of, or in the course of, employment and the employment is a significant contributing factor to the aggravation—
 - (i) a personal injury;
 - (ii) a disease;
 - (iii) a medical condition, if the condition becomes a personal injury or disease because of the aggravation;

...

- (4) For subsection (3)(b), to remove any doubt, it is declared that an aggravation mentioned in the provision is an injury only to the extent of the effects of the aggravation.

[3] This appeal to the Commission is by way of a hearing de novo in which the onus of proof falls upon the Appellant.¹ The Commission must be satisfied that the worker has suffered an injury within the meaning of the Act.²

[4] The Appellant must prove, on the balance of probabilities,³ that the claim is one for acceptance. That is:

- (a) the Appellant suffered an injury;
- (b) the injury arose out of, or in the course of, his employment with the Employer; and
- (c) the employment with the Employer was a significant contributing factor to the injury.

[5] After careful consideration of all of the evidence, I find that on the balance of probabilities the Appellant did not suffer an injury within the meaning of the term pursuant to s 32 of the Act.

¹ *Rossmuller v Q-COMP* [2010] ICQ 4

² *McDonald v Q-COMP (No. 2)* (2008) 188 QGIG 180; *Qantas Airways Limited v Q-COMP* (2006) 181 QGIG 301

³ *Briffa v Q-COMP* (2005) 180 QGIG 70

[6] My reasons follow.

Background

[7] The Appellant is 65 years old and has been a truck driver most of his working life.

[8] In January 2018, the Appellant commenced employment as a truck driver for Jarbie Pty Ltd, trading as Kalbar Bobcat Hire ('Kalbar') working as a driver on a casual basis.

[9] The Appellant was primarily engaged to drive a tipper truck back and forth from quarries to various sites along with loading and unloading gravel and road base.

[10] The Appellant drove three trucks described as the Hino, the Isuzu, and the International Acco. At the beginning of his employment the Appellant primarily drove the Hino truck before shifting to the International Acco truck.

[11] The Appellant made a successful workers' compensation claim to WorkCover after he sustained an injury to his left elbow in a work-related incident on 13 November 2018. WorkCover determined that the Appellant injured his left elbow after hitting it on the gear stick whilst driving his truck. WorkCover did not accept the Appellant's claim regarding his left shoulder noting the existence of a pre-existing degenerative condition. The Appellant did not return to work for Kalbar after this injury.

[12] On 12 June 2020, the Appellant lodged an application for workers' compensation for an alleged injury sustained to his left shoulder 'over time in 2018' whilst employed with Kalbar.

Appellant's Statement of Facts and Contentions

[13] The Appellant's Statement of Facts and Contentions ('SOFC') are summarised as follows -

- a) The Appellant sustained a personal injury, namely, an injury to his left shoulder described as 'adhesive capsulitis / rotator cuff tendinopathy with small intrasubstance tear and subacromial bursitis', consistent with the finding made by the Respondent in the decision appealed against.
- b) The injury arose out of, or in the course of his employment with Kalbar.
- c) The injury represented an ongoing aggravation of a pre-existing asymptomatic degenerative condition in the Appellant's left shoulder evidenced by the onset of symptoms in or about August 2018, which arose after approximately 10

months of strenuous physical activities carried out by him on a daily basis upon the faulty tailgate of his work truck.

- d) The medical evidence shows that these symptoms would not have occurred when they did, if at all, but for the aggravating effect of the strenuous physical work carried out by the Appellant on the faulty tailgate.
- e) Contrary to the finding made by the Respondent in the decision appealed against, the Appellant's employment with Kalbar was a significant contributing factor to the injury.
- f) Contrary to the finding made by the Respondent in the decision appealed against, the Appellant sustained an injury in accordance with section 32(1) of the Act and so has an entitlement to compensation thereunder.

Respondent's Statement of Facts and Contentions

[14] The Respondent submits the following contentions –

- (a) The Appellant was a 'worker' within the meaning of the Act.
- (b) The Appellant suffers from an underlying degenerative process of the left shoulder.
- (c) That injury was not caused by the Appellant being "*forced to manually open and close the faulty tailgate on his truck with the aid of a crowbar and sledgehammer*" between 1 February 2018 and 13 November 2018, and therefore did not arise out of, or in the course of, his employment with Kalbar.
- (d) The Appellant has not suffered a compensable injury under the Act.

Consideration

[15] The parties are in agreement that the Appellant was a worker within the meaning of s 11 of the Act and that the Appellant suffers from a personal injury being a degenerative deterioration of his left shoulder. The Appellant contends that he suffered an injury by way of an aggravation of the degenerative condition in his left shoulder over a period of 10 months of manually handling a faulty tailgate during his time driving the International Acco truck at work for Kalbar. The Respondent contends that the Appellant's injury did not arise out of or in the course of his employment and that his employment cannot be a

significant contributing factor to any injury.

[16] The issues to be determined in this matter are the following –

- a) whether the Appellant suffered an injury by way of an aggravation of an underlying degenerative condition during the period contended, that is between 1 February 2018 and 13 November 2018;
- b) if so, whether that injury arose out of or in the course of the Appellant's employment; and
- c) if the Appellant did suffer an injury that arose out of or in the course of his employment, whether the employment was a significant contributing factor to the injury.

[17] The first issue to be considered is whether the Appellant suffered an injury by way of an aggravation during the period contended, that is between 1 February 2018 and 13 November 2018.

[18] The Appellant had previously lodged a claim with WorkCover for an injury to his left elbow and left shoulder. The Appellant described the mechanism of injury as occurring when his hand slipped after his truck hit a pothole, resulting in his elbow hitting the gear stick on 13 November 2018. This claim was accepted as an injury to the left elbow arising out of the incident ('the gear stick incident').

[19] The claim for the left shoulder injury was not accepted by WorkCover on the basis that the medical evidence did not support the claim that a shoulder injury had been sustained as a result of the Appellant striking his elbow on the gear stick.

[20] The Appellant contacted WorkCover on 10 June 2020 to advise that his shoulder symptoms were related to issues with closing the tailgate on his work truck⁴ and sought advice as to whether a second claim may be submitted.

[21] The Appellant attended a consultation with Dr Chowdury on 11 June 2020 and referred to his left shoulder pain in relation to the regular work of manually using a crowbar and his shoulder to operate a faulty tailgate on his work truck ('the tailgate issue').⁵ The Appellant then made another claim on 16 June 2020 for a left shoulder injury arising over the period of 1 February 2018 to 13 November 2018 ('the relevant period').

Appellant's evidence

⁴ Exhibit 1, Trial Bundle, WorkCover Queensland Communications Report dated 10 June 2020 [302].

⁵ Exhibit 1, Trial Bundle, Records of Fassifern Medical Centre [117].

- [22] The Appellant gave evidence about the tailgate issue, stating that he had to manually use a crowbar and his shoulder to lift and push the tailgate over a period of ten months.
- [23] The Appellant gave evidence that his left shoulder pain had been present while working on the tailgate and not after the gear stick incident.⁶ The Appellant's evidence was that the symptoms had become progressively worse over time and more problematic once the ulnar nerve issue associated with the elbow symptoms had subsided.⁷
- [24] Under cross-examination, the Appellant denied that he was experienced in making workers' compensation claims and knew what to do in order to make a claim. This was despite the Appellant's evidence that he had made a previous workers' compensation claim regarding his right shoulder in 2016 in relation to a different employer. The Appellant gave evidence about a right shoulder claim in 2016 relating to an aggravation to his right shoulder whilst working on his work truck.⁸
- [25] The Appellant indicated that his left elbow claim commenced after he consulted a doctor⁹ and it was not until he consulted with his physiotherapist, Ms Jen, that he recognised that he had suffered a distinct injury to his left shoulder.
- [26] The Appellant denied that his shoulder pain was a recent invention and gave evidence that the elbow symptoms had masked the pre-existing left shoulder pain.¹⁰
- [27] The Appellant's evidence regarding the commencement of his shoulder pain was that his shoulder started to get sore after two or three months of pushing the tailgate¹¹ and that he mentioned the left shoulder pain in connection with the tailgate issue to Dr Faridi and other medical professionals. The Appellant stated that it was not documented by these medical professionals due to their focus on his elbow.
- [28] The Appellant gave evidence that his left shoulder pain had been present while working on the tailgate before the gear stick incident of 13 November 2018 and that the symptoms became progressively worse over time and more problematic once the ulnar nerve issue had subsided.¹²
- [29] When the Appellant made the workers' compensation claim in 2018 for an injury to his left

⁶ T 1 – 49, ll 26 – 28.

⁷ T 1 – 49, ll 26 – 30.

⁸ T 1 – 22, ll 19 – 20; T1 – 24, ll 5 – 6.

⁹ T 1 – 25, ll 39 – 41.

¹⁰ T 1 – 52, ll 26 – 30.

¹¹ T 1 – 10, ll 37 – 39.

¹² T 1 – 49, ll 26 – 30.

shoulder, he considered that the symptoms were a result of the gear stick incident. If it were the case that the Appellant had struggled daily with the tailgate mechanism and had suffered shoulder pain in the months prior to the gear stick incident, it is in my view unlikely that these issues would not have been mentioned at this time.

Appellant's disclosure of symptoms

- [30] After the Appellant sustained an injury to his left elbow following the gear stick incident,¹³ he consulted Dr Faridi who sent the Appellant for medical tests. The Appellant saw Dr Faridi again four weeks following the injury, with Dr Faridi assisting him to lodge a WorkCover claim with respect to his left elbow injury. Once Dr Faridi verified the nature of the left elbow injury, the Appellant informed his manager, Mr Goetsch.¹⁴
- [31] The Appellant gave evidence that he told his general practitioner Dr Faridi about his left shoulder symptoms in August 2018 and that Dr Faridi agreed with his suggestion that he may have pulled a muscle.¹⁵
- [32] Dr Faridi worked at the Fassifern Medical Centre throughout the relevant period. The records of the Fassifern Medical Centre indicate that the first recorded time that the Appellant reported left shoulder pain to a medical practitioner was on 11 June 2020.¹⁶
- [33] The Appellant gave evidence that he told a number of other people that he was experiencing left shoulder pain, including Mr Goetsch, Mr Mears, and other workers.¹⁷
- [34] Mr Mears, a worker who worked on the same sites as the Appellant, gave evidence that he saw the Appellant use his left shoulder to lift the tailgate flap five to eight times per day¹⁸ and that he would observe the Appellant and could see the pain on his face. Mr Mears stated that he would speak to the Appellant after work and the Appellant would tell him about the pain he felt in his left shoulder.¹⁹
- [35] Mr Goetsch, the Appellant's employer, denied that the Appellant informed him that his arm was 'killing him' from being required to jimmy and sledgehammer the tailgate.²⁰ It is implausible that the Appellant did not report the symptoms to medical practitioners on the basis that his shoulder symptoms were 'minor', as submitted by the Appellant, and yet have

¹³ T 1 – 6, ll 14 – 20.

¹⁴ T 1 – 27, ll 30 – 39.

¹⁵ T 1 – 11, ll 1 – 6; T1 – 53, ll 6 – 10.

¹⁶ Exhibit 1, Trial Bundle, Records of Fassifern Medical Centre [117].

¹⁷ T 1 – 11, ll 12 – 40.

¹⁸ T 1 – 64, l 38 to T1 – 65, l 4.

¹⁹ T 1 – 65, ll 30 – 33; T1 – 77, ll 12 – 15.

²⁰ T 3 – 20, ll 28 – 29.

reported that his shoulder was 'killing him' to Mr Goetsch during the same period.²¹

- [36] Similarly, Mr Mears' evidence that he saw the Appellant 'wince' in pain on site and had discussions with him about his left shoulder symptoms is incongruent with the lack of documented reporting of the symptoms to medical practitioners. I do not accept the Appellant's submission that Mr Mears had 'no axe to grind',²² with the evidence indicating difficulties in the relationship between Mr Mears and Mr Goetsch arising from disputed property matters. Consequently, I approach the evidence of Mr Mears with some caution.
- [37] Mr Andrew Burns was employed as a machine operator for Kalbur for approximately ten years. He gave evidence that he was not aware that there had been a problem with the tailgate and did not recall that Mr Mears had complained about the trucks at Kalbur.
- [38] WorkCover referred the Appellant to Dr O'Toole in early 2019 regarding his left elbow injury. The Appellant gave evidence that he told Dr O'Toole about the problems he had experienced with the tailgate on his work truck and queried whether there might be another mechanism to his left arm problems, namely working on the tailgate.²³ The Appellant stated that he told Dr O'Toole that he had pain all the way down his left arm which included left shoulder pain²⁴ and that the pain connected with the pins and needles in his left elbow was most prominent at that time.²⁵
- [39] The Appellant gave evidence that Dr O'Toole told him that he had sustained more damage to his left arm than just the ulnar nerve²⁶ before referring him for an MRI of his left elbow.²⁷
- [40] WorkCover then referred the Appellant to see Dr Scott. The Appellant gave evidence that he explained to Dr Scott the details regarding the faulty tailgate²⁸ and told Dr Scott that he was experiencing left shoulder pain.²⁹ The Appellant stated that Dr Scott examined his left shoulder³⁰ and conducted a left arm flexion test during which his shoulder became painful.³¹
- [41] The Appellant gave evidence that when he saw Dr Melsom he told him about the tailgate

²¹ T 3 – 20, ll 28 – 29.

²² Appellant's submissions filed 10 January 2023 [193].

²³ T 1 – 44, ll 25 – 28.

²⁴ T 1 – 14, ll 12 – 14.

²⁵ T 1 – 44, ll 33 – 36.

²⁶ T 1 – 14, ll 25 – 26.

²⁷ T 1 – 14, l 30.

²⁸ T 1 – 14, ll 44 – 46.

²⁹ T 1 – 14, ll 5 – 10.

³⁰ T 1 – 15, ll 12 – 17.

³¹ T 1 – 15, ll 19 – 21.

issue and his left elbow symptoms including the pins and needles he had experienced with the ulnar nerve.³² The Appellant stated that he had mentioned his shoulder symptoms to Dr Melsom, but his emphasis at that time was on his left elbow.³³

- [42] The Appellant stated that Dr Melsom ordered an MRI on the Appellant's left elbow, and he was tentatively booked in for elbow surgery which was ultimately not required as the pins and needles abated.³⁴
- [43] The Appellant saw Dr Johnstone on two occasions for his left elbow symptoms and gave evidence that he told Dr Johnstone at the first appointment about the tailgate and his left shoulder symptoms.³⁵ At the appointment on 22 May 2020 the Appellant stated that he informed Dr Johnstone that his left shoulder felt 'dislocated' and whilst the symptoms had been present the entire time, they had become more prominent after he started going to physiotherapy.³⁶
- [44] The Appellant was referred by WorkCover to see Ms Heidi Jen, physiotherapist, on 25 March 2020 for treatment of his left elbow symptoms.³⁷ The Appellant gave evidence that he told Ms Jen about his left shoulder pain.³⁸ After suffering 'agony' during Ms Jen's conduction of an abduction test on his left shoulder, the Appellant told Ms Jen that he had injured his shoulder at the same time that he had injured his elbow.³⁹ Ms Jen advised WorkCover on 31 March 2020 that the Appellant had informed her that he had shoulder pain since the elbow injury.⁴⁰
- [45] If it were the case that the Appellant was suffering painful symptoms in his left shoulder months prior to the gear stick incident as claimed, it is unlikely that he would have continued to use his shoulder when manipulating the tailgate. In circumstances where the tailgate mechanism was causing shoulder symptoms as described by the Appellant, one would expect that the Appellant would have sought medical assistance to address the pain.
- [46] The Appellant produced statutory declarations from the following people relating to his left shoulder injury -

- Mr Gray Mears, dated 15 December 2020;

³² T 1 – 16, ll 18 – 21; T 1 – 16, ll 35 – 40.

³³ T 1 – 16, l 35 to T 1 – 17, l 6.

³⁴ T 1 – 17, ll 16 – 23.

³⁵ T 1 – 17, ll 44 – 48; T 1 – 50, ll 12 – 13.

³⁶ T 1 – 49, ll 40 – 41; T 1 – 49, ll 45 – 48.

³⁷ Exhibit 1, Trial Bundle, Report of Ms Heidi Jen to Dr Melsom dated 25 March 2020 [234].

³⁸ T 1 – 49, l 16.

³⁹ T 1 – 19, l 48 to T 1 – 20, l 5.

⁴⁰ Exhibit 1, Trial Bundle, WorkCover Queensland Communications Report dated 31 March 2020 [316].

- Dr Imran Faridi, dated 10 June 2021; and
- The Appellant, dated 26 November 2020.

[47] In his statutory declaration dated 10 June 2021, Dr Faridi stated that by reference to the medical notes it can be said that the Appellant was complaining of left arm symptoms referable to his repetitive heavy work on the tailgate of his work truck as far back as February 2019. This note related to 'biceps tendonitis'. The statutory declaration does not mention where this reference is to be found in the medical notes, however, the Respondent submits that it is a reference to the first note of the mechanism to Dr O'Toole in relation to the left elbow injury. Dr Faridi proceeds to state the following in his statutory declarations

—

I do not have any independent recollection of Mr Richards mentioning that he was suffering symptoms to his left shoulder back in August 2018. There does not appear to be any record of this in the medical notes.⁴¹

[48] The Appellant gave evidence that he told Dr Scott,⁴² Dr Johnstone,⁴³ and Dr Melsom⁴⁴ that he had shoulder pain when he attended upon them. Notably, left shoulder pain was not mentioned in the reports of these practitioners.

[49] The difficulty for the Appellant's case is that there was no reporting of a left shoulder injury at any time prior to 13 November 2018. The Appellant gave evidence that Dr Faridi and Dr O'Toole failed to note his left shoulder pain despite his reporting the pain to them.⁴⁵ The Appellant maintained that he had reported his left shoulder pain to Dr Scott, Dr Johnstone, and Dr Melsom despite there being no reference to shoulder pain in their reports. The Appellant submits that it is unsurprising that the reported left shoulder pain was not recorded if it was 'minor, deemed not clinically significant or not contributing to his elbow problem'.⁴⁶ I find it unlikely that not a single medical practitioner made any reference to the left shoulder pain if the Appellant mentioned it at all. It is also inconsistent with the Appellant's evidence that he told Dr Faridi that "*I've got really bad - I've got really bad pain up here*" in reference to his shoulder.⁴⁷ Such a statement could not reasonably be described as minor or not clinically significant enough for a medical practitioner to make a record of it.

⁴¹ Exhibit 1, Trial Bundle, Statutory declaration of Dr Faridi dated 10 June 2021 [40].

⁴² T 1 – 47, ll 1 – 12.

⁴³ T 1 – 50, ll 12 – 13.

⁴⁴ T 1 – 47, ll 32 – 35.

⁴⁵ T 1 – 44, ll 30 – 41; T1 – 45, ll 4 – 7.

⁴⁶ Appellant's written submissions filed 10 January 2023 [93].

⁴⁷ T 1 – 11, l 3.

- [50] Whilst I note Dr O'Toole's evidence that '*pain is purely subjective*',⁴⁸ in the usual course of events it will be expected that someone who has begun to feel symptoms of pain will advise their medical practitioner. It is particularly unusual for a person to not mention such pain when they are regularly seeing medical practitioners for other matters.

Medical evidence

- [51] Dr Faridi was the General Practitioner at Fassifern Medicare Centre who had first seen the Appellant on 23 November 2018 for treatment of his left elbow injury following the gear stick incident.

- [52] In a statutory declaration dated 26 November 2020, the Appellant swore the following –

I do however specifically recall mentioning my left shoulder symptoms to my General Practitioner Dr Faridi in or about August 2018. Unfortunately, he did not write it down in his notes. My subsequent enquiries of him overtime indicate that he had some recollection that it was mentioned by me.⁴⁹

- [53] In his examination in chief, the Appellant gave the following evidence –

I spoke to Dr Faridi in August. That was my first visit to Dr Faridi about my shoulder, because I wanted to know what was going on with it.⁵⁰

...

When I spoke to Dr Faridi I said to him 'I've got really bad - I've got really bad pain up here' and I said 'it feels like I've pulled my muscle' and he said 'Peter you probably have with what you were doing' because he asked me what I was doing and I explained to him exactly how the accident.- or what was happening with that tailgate.⁵¹

- [54] The Appellant's evidence is not consistent with Dr Faridi's contemporaneous documentary evidence, nor with the statutory declaration provided by Dr Faridi.

- [55] In Dr Faridi's statutory declaration he stated the following –

I do not have any independent recollection of Mr Richards mentioning that he was suffering symptoms to his left shoulder back in August 2018. There does not appear to be any record of this in the medical notes.⁵²

⁴⁸ T 2 – 36, ll 22 – 24.

⁴⁹ Exhibit 1, Trial Bundle, Statutory Declaration of the Appellant dated 26 November 2020 [34].

⁵⁰ T 1 – 10, ll 46 – 48.

⁵¹ T 1 – 11, ll 1 – 6.

⁵² Exhibit 1, Trial Bundle, Statutory Declaration of Dr Faridi dated 10 June 2021 [40].

[56] Dr Faridi's statutory declaration also included the following –

By 11 May 2020 when I saw Mr Richards he had received a specialist diagnosis of left shoulder supraspinatus tear. There is a file note made by me on 11 May 2020 in which there is mention of his left shoulder injury. Mr Richards indicated to me at that time that the left shoulder pain had been present since August 2018.⁵³

[57] Dr Faridi noted in his statutory declaration that his review of the Fassifern Doctors clinical notes indicated that the Appellant was complaining of left arm symptoms in February 2019, referring to a file note made by Dr John Wang.

[58] The clinical note by Dr Wang is dated 19 February 2019 and states the following –

Letter from wcover
cover for aggravation of L arm tendinopathy
...⁵⁴

[59] At the hearing Dr Faridi gave evidence that if there was no documentation, he could not definitely say whether the Appellant mentioned his left shoulder injury to him in 2018 or not.⁵⁵

[60] Dr Faridi confirmed that he had no recollection of the Appellant mentioning left shoulder pain in August 2018. In response to a question as to what he would have done had the Appellant reported acute shoulder pain, Dr Faridi stated "*I would have definitely examined his shoulder*".⁵⁶ He went on to describe the types of investigation he would typically do on a person complaining of acute shoulder pain, being x-rays in the case of a fall injury or ultrasounds,⁵⁷ and stated that it was unlikely that the Appellant's left shoulder would have been a major problem at the time if it was not being investigated.⁵⁸

[61] Dr Faridi gave evidence under cross-examination that the left shoulder concerns raised by the Appellant might not have been clinically relevant at the time and so may not have been recorded, as general practitioners generally concentrate on the active issues.⁵⁹ Dr Faridi's evidence was that if the left shoulder problems were not active issues, he may not have taken a note.⁶⁰ Further, Dr Faridi said it was unlikely that the shoulder was a major

⁵³ Exhibit 1, Trial Bundle, Statutory Declaration of Dr Faridi dated 10 June 2021 [40].

⁵⁴ Exhibit 1, Trial Bundle, Records of the Fassifern Medical Centre [124].

⁵⁵ T 1 – 57, ll 4 – 16.

⁵⁶ T 1 – 59, ll 20 – 21.

⁵⁷ T 1 – 59, ll 23 – 25.

⁵⁸ T 1 – 59, ll 27 – 36.

⁵⁹ T 1 – 60, ll 19 – 23.

⁶⁰ T 1 – 60, ll 25 – 27.

problem on 23 November 2018 if it was not recorded.⁶¹

- [62] The Appellant attended upon the General Practitioners at Fassifern Doctors on nineteen occasions between 1 February 2018 and 13 November 2018,⁶² and yet there is no record that he reported pain in his left shoulder during this time. It seems to me unlikely that doctors would fail to record even a brief note of symptoms as outlined by the Appellant. In these circumstances it appears more likely than not that left shoulder pain was not reported by the Appellant.
- [63] The documentary evidence weighs heavily toward a conclusion that the Appellant did not report left shoulder pain to Dr Faridi in or about August 2018. There is simply no contemporaneous evidence that the Appellant reported left shoulder pain at this time.
- [64] To be clear, the Appellant did discuss his right shoulder injury with the general practitioners during that time and he had a successful claim for a right shoulder injury relating to different circumstances arising in previous employment.⁶³
- [65] The Fassifern Medical Centre records indicate that between August 2018 and March 2020 the Appellant attended upon the general practitioners at least forty times for a range of matters.⁶⁴ Other than the note made outside of the relevant period by Dr Wang of 19 February 2019 referring to 'left arm tendinopathy', of which there was no follow up, there is no record of the Appellant having reported left shoulder pain during any of those appointments.
- [66] The evidence supports a finding that there was no reporting of the Appellant's left shoulder symptoms to a general practitioner between 1 February 2018 and 13 November 2018.

Dr Melsom (hand and upper limb surgeon)

- [67] Dr Melsom was the Appellant's treating orthopaedic surgeon who provided reports on the following dates –

- 8 November 2019
- 5 December 2019
- 3 February 2020
- 29 April 2020

⁶¹ T 1 – 59, ll 30 – 36.

⁶² Exhibit 1, Trial Bundle, Records of the Fassifern Medical Centre [116].

⁶³ Exhibit 1, Trial Bundle, Records of the Fassifern Medical Centre dated 27 February 2018 [129]; Exhibit 1, Trial Bundle, Clinical Notes of PhysioActive [220].

⁶⁴ Exhibit 1, Trial Bundle, Records of the Fassifern Medical Centre [116].

- 29 April 2020
- 9 June 2020
- Work Capacity Certificate 18 June 2020
- 23 March 2021

[68] The Appellant contends that the focus during the period prior to November 2019 was on the pain in his left elbow.⁶⁵

[69] In response to a question as to whether severe pain in one location could take priority or the focus over other pain, Dr Melsom stated that whilst such pain could be a distracting influence it would not remove all of the pain from elsewhere.⁶⁶

[70] The focus of the Appellant and the doctors was understandably on the left elbow as that was the subject of the Appellant's complaint of pain. However, it seems implausible that the Appellant also felt pain in his left shoulder but did not mention it during any of the numerous medical appointments with Dr Melsom.

[71] Moving on to 2020, Dr Melsom examined the Appellant again on 29 April following his report of left shoulder pain. In the report Dr Melsom provided to Dr Faridi dated 29 April 2020, he stated -

Peter is insistent that his left shoulder was injured at the same time as his left elbow and forearm. His symptoms have been quite significant and he is taking analgesics on a regular basis for both his elbow and his shoulder.

The problem is that there is no documented injury in any of the GP notes that I have available from Dr John Scott's Independent Medical Examination or Dr O'Toole's examination and report.

This does not bode well for Peter having this accepted as a WorkCover claim. This kind of patterning that I am seeing on the MRI scan could be any number of 50 year old manual workers and these findings are not specific to any particular injury.

...

It is not really possible to ascribe a specific work related event to his shoulder pain or a shoulder injury from any of the records that I have available or any of the reports that have been done before.⁶⁷

[72] Dr Melsom reported the following with respect to the Appellant's 'abnormal pain behaviour' –

Clinically Peter did display some abnormal pain behaviour on examining his shoulder, with gripping

⁶⁵ T 1 – 15, ll 9 – 10; T 1 – 16, ll 35 – 42; T 1 – 19, ll 18 – 24; and T 1 – 21, ll 3 – 17; Exhibit 1, Trial Bundle, Statutory Declaration of the Appellant filed 26 November 2020 [36].

⁶⁶ T 2 – 9, ll 35 – 42.

⁶⁷ Exhibit 1, Trial Bundle, Report of Dr Melsom to Dr Faridi dated 29 April 2020 [212].

his shoulder with his right hand continuously and preventing me from examining him effectively.⁶⁸

[73] Dr Melsom examined the Appellant again with respect to his left shoulder on 25 June 2020. Dr Melsom concluded in his subsequent report that he could not state that the Appellant had a work-related injury to his left shoulder and that he agreed with Dr Johnstone's report.⁶⁹

[74] On 23 March 2021 Dr Melsom provided a further report noting the following with respect to the Appellant's MRI –

It is not possible to say whether this diagnosis was present in 2018, but I had no reason to believe so as this would have caused very demonstrable problems with the shoulder necessitating an examination from his GP and it is unlikely that this would have passed by unnoticed as a clinical diagnosis.

However, many of the changes seen on the MRI scan could be ascribed to age related degenerative changes, which do occur and would be found in a number of people and in particular manual workers who were to have an MRI of their shoulders whether symptomatic or asymptomatic.

...⁷⁰

[75] Dr Melsom proceeded to make the following observations regarding Dr Faridi's statutory declaration –

A colleague of Dr Faridi, Dr Chowdhury, saw the claimant on the 27th March 2020 and he was then complaining of left shoulder pain. The claimant mentioned to Dr Chowdhury on the 11th June 2020 that this injury to the left shoulder had occurred as a separate injury working on the tailgate of the work truck separate from the left elbow injury. In paragraph 15, while Dr Faridi has agreed that this could be a cause of the left shoulder injury unfortunately there still remains a lack of objective contemporaneous notes to that effect.⁷¹

[76] In his report Dr Melsom outlined the following conclusion –

While I think it is not unreasonable that the injury had occurred in August 2018 in the event described by the claimant and given some of the circumstantial evidence surrounding this it is certainly possible, I cannot definitively say that this is the case without clear evidence that this had occurred on this particular date. However, my feeling is that the claimant is likely telling the truth in this regard, and while I think it is certainly feasible and possible that this has resulted in a left shoulder injury, there remains a lack of objective evidence to this effect specifying this particular period of time. [emphasis added]⁷²

⁶⁸ Ibid.

⁶⁹ Exhibit 1, Trial Bundle, Report of Dr Melsom dated 25 June [101].

⁷⁰ Exhibit 1, Trial Bundle, Report of Dr Melsom to Dr Faridi dated 29 April 2020 [111].

⁷¹ Ibid.

⁷² Ibid.

- [77] Dr Melsom states that the Appellant's degenerative changes 'could' have been aggravated by his description of the tailgate activities, but noted the following –

However, it is somewhat unusual that he did not complain of pain in many subsequent consultations, and this was not documented in written form or that any examination of the left shoulder was performed during this period.⁷³

- [78] In his report of 23 March 2021, Dr Melsom refers to Dr Scott's report of 13 May 2019 in which he observed that the Appellant pointed to the location of the pain as being his upper arm (i.e. brachium). Dr Melsom gave evidence that Dr Scott's reference to 'brachium' was the area between the elbow and the shoulder.⁷⁴ The Appellant submits that this is evidence of when the Appellant made a complaint about the tailgate and left shoulder pain, noting that Dr Melsom observed that this pain generally would arise from the shoulder and not the elbow.⁷⁵ I note that Dr Scott's reference to the 'brachium' is not pursued further. Dr Melsom gave evidence that shoulder pain is commonly felt in the brachium and that elbow pathology does not generally radiate to the middle of the upper arm.⁷⁶

- [79] Dr Melsom noted in his report that it was necessary to observe symptoms before attributing the tailgate mechanism as a cause of an aggravation injury. Dr Melsom outlined the following –

Having read through the statutory declaration by Dr Faridi it is apparent that while Mr Richards has stated that he told Dr Faridi that he had left shoulder symptoms in August 2018 there is no objective record in Dr Faridi's notes that this was the case and, therefore, it is somewhat supposition that the injury occurred on that date without any objective evidence.⁷⁷

- [80] The Appellant gave evidence that he mentioned the tailgate problems and his left shoulder symptoms to Dr Melsom, but that his emphasis at the time was fixing his left elbow and restoring feeling back into his lower arm.

- [81] I accept the Respondent's submission that doctors make clinical notes of reported pain. Where there is no clinical record of reported pain, it is reasonable to conclude that no pain was reported at that time.

Dr O'Toole, occupational and environmental physician

- [82] Dr O'Toole examined the Appellant on 1 February 2019 with respect to his left elbow

⁷³ Exhibit 1, Trial Bundle, Report of Dr Melsom to Dr Faridi dated 29 April 2020 [113].

⁷⁴ T 2 – 7, ll 32 – 33.

⁷⁵ Exhibit 1, Trial Bundle [109].

⁷⁶ T 2 – 9, ll 9 – 13.

⁷⁷ Exhibit 1, Trial Bundle, Report of Dr Melsom dated 23 March 2021 [110].

injury. Dr O'Toole produced a report noting that the Appellant referred to the tailgate issues in reference to his left elbow injury.⁷⁸ Despite the Appellant's contention that he mentioned the left shoulder symptoms to Dr O'Toole,⁷⁹ the report of 1 February 2019 and a subsequent report of 15 March 2019 make no reference to a report of left shoulder pain.⁸⁰

[83] In the Appellant's statement of facts and contentions he outlined the following –

Dr O'Toole in consultation suggested to the Appellant that he may have sustained further damage above the elbow including to his left shoulder but as that was not the subject of his left elbow Workcover claim no reference was made to it in his report.⁸¹

[84] I accept the Respondent's submission that this matter was not put to Dr O'Toole in cross-examination and consequently afford this contention limited weight.

[85] In Dr O'Toole's report he outlined the following –

The development of common extensor origin tendinopathy is a degenerative and an insidious one, due to lack of vascular supply and altered structure within the common extensor tendon complex.

It is common for this condition to be episodic in nature. As each episode progresses, symptoms increase their frequency and intensity.⁸²

[86] Dr O'Toole gave the following evidence under cross-examination –

So when one of those episodes - or 'flares', I guess, is another way of determining it - occurs, if you're using your forearm muscles, then you're going to have to have symptoms because the area that is being handled by the body or trying to be repaired by the body is actively involved in the muscles that you're trying to use. So it's not that the actions of the muscles are making the condition worse or flaring up the actual condition; it's that the pre-existing flare-up of the condition makes movement worse. To use an analogy, if I fell over and broke my leg playing football and then I went to walk on that leg the next day at work, the action of walking at work would cause pain and discomfort, but it didn't cause the broken leg.⁸³

[87] The Appellant gave evidence that he told Dr O'Toole that he had pain all the way down his left arm which included left shoulder pain⁸⁴ and Dr O'Toole told the Appellant that he had sustained more damage than his ulna nerve.⁸⁵

⁷⁸ Exhibit 1, Trial Bundle, Report of Dr O'Toole dated 1 February 2019 [42].

⁷⁹ Exhibit 1, Trial Bundle, Appellant's Statement of Facts and Contentions dated 12 May 2021 [25].

⁸⁰ Exhibit 1, Trial Bundle, Report of Dr O'Toole dated 15 March 2019 [50].

⁸¹ Exhibit 1, Trial Bundle, Appellant's Statement of Facts and Contentions dated 12 May 2021 [26].

⁸² Exhibit 1, Trial Bundle, Report of Dr O'Toole dated 15 March 2019 [47].

⁸³ T 2 – 33, ll 20 – 29.

⁸⁴ T 1 – 14, ll 12 – 14; T 1 – 44, ll 19 – 20.

⁸⁵ T 1 – 14, ll 24 – 26.

- [88] The fact that Dr O'Toole referred the Appellant for an MRI on his left elbow only, and sought no imaging on the left shoulder, suggests that it is unlikely that the Appellant advised Dr O'Toole that he had symptoms in his shoulder.
- [89] The Appellant submits that little weight ought to be given to Dr O'Toole's opinion as it is unclear whether Dr O'Toole relied upon the 'beyond reasonable doubt' standard of proof when addressing a hypothetical medical causation question in cross examination⁸⁶ when dealing with causation issues in his reports.
- [90] In response to a hypothetical question in cross-examination, Dr O'Toole replied, "*I don't believe that that is sufficient information for me to determine beyond a reasonable doubt that that caused an aggravation*".⁸⁷ After prompting by Counsel, Dr O'Toole then replied "*Then I replace the term 'beyond reasonable doubt' with 'more likely than not'. There's insufficient information on a time scale that would allow me to make that determination.*"⁸⁸
- [91] I am not persuaded that the use of the term 'beyond reasonable doubt' in response to a hypothetical question in cross-examination provides sufficient grounds to assume that Dr O'Toole adopted that standard of proof in his medical reports.

Dr Scott – Orthopaedic surgeon

- [92] The Appellant's evidence was that during the appointment with Dr Scott he was experiencing left shoulder pain and during the left arm flexion test his left shoulder became painful.⁸⁹
- [93] Dr Scott examined the Appellant on 13 May 2019 and produced a report stating that the Appellant had reported upper arm pain as a result of having to use a crowbar to lever up the tailgate. Dr Scott described the Appellant's current symptoms as –

His current symptoms are an inability to straighten his left elbow fully, and around the medial elbow. He also feels pain in the lateral elbow and the extensor muscle mass.⁹⁰

- [94] Dr Scott's report was tendered into evidence by consent; however, he was unavailable to be cross-examined due to illness.
- [95] In Dr Scott's report he notes that the Appellant's "*right shoulder has an excellent pain free*

⁸⁶ T 2 – 35, ll 14 – 35.

⁸⁷ T 2 – 35, ll 34 – 35.

⁸⁸ T 2 – 35, ll 39 – 41.

⁸⁹ T 1 – 15, ll 19 – 21.

⁹⁰ Exhibit 1, Trial Bundle, Report of Dr Scott dated 13 May 2019 [58].

range of movement".⁹¹ I accept the Respondent's submission that it is reasonable to assume that the reference to 'right shoulder and elbow' in that paragraph was an error given that the Appellant had attended due to symptoms associated with his left arm. As Dr Scott was unable to be cross-examined this proposition could not be put to him. I note that Dr Scott determined after examining the Appellant's presumably left shoulder that he had an excellent pain-free range of movement notwithstanding the Appellant's report of upper arm symptoms resulting from using the crowbar on the tailgate.

[96] Dr Scott provided a report to WorkCover on 14 June 2019 stating that "*using a crowbar to jimmy a tailgate open, as described to me, could in principle lead to tennis elbow or an aggravation of pre-existing degenerative pathology*".⁹²

[97] Dr Scott's statement that the tailgate action could 'in principle' lead to an aggravation of a pre-existing degenerative condition appears to have been in relation to queries regarding the Appellant's elbow injury. In Dr Scott's report of 13 May 2019, he states, "*Peter Richards has left elbow symptoms*" and that the Appellant "*has ongoing incapacity because of left elbow pain*".⁹³ This statement could not reasonably be relied upon to determine that the tailgate mechanism resulted in an aggravation to the Appellant's shoulder.

Dr Johnstone – orthopaedic surgeon

[98] Dr Johnstone examined the Appellant on 22 November 2019. In his subsequent report Dr Johnstone states that the Appellant referred to the tailgate issues in relation to his left forearm extensor muscles and not his left shoulder. The Appellant claims that he mentioned his left shoulder pain to Dr Johnstone and that Dr Johnstone failed to note it in his report.⁹⁴

[99] The Appellant gave evidence that when he saw Dr Johnstone again on 22 May 2020, he informed him that his left shoulder felt like it was 'dislocated'⁹⁵ and advised that while his left shoulder pain had been present the entire time it had become more prominent after he started going to physiotherapy.

[100] The Appellant's evidence was that Dr Johnstone flexed his left arm in the forward plane and again when his arm reached shoulder height, he started experiencing left shoulder pain.⁹⁶

⁹¹ Ibid.

⁹² Exhibit 1, Trial Bundle, Dr Scott's letter to WorkCover dated 14 June 2019 [61].

⁹³ Exhibit 1, Trial Bundle, Report of Dr Scott dated 13 May 2019 [59].

⁹⁴ T 1 – 18, ll 45 – 48; T 1 – 19, ll 1 – 10; T 1 – 50, ll 9 – 13.

⁹⁵ T 1 – 49, ll 40 – 41.

⁹⁶ T 1 – 18, l 45 to T 1 – 19, l 10.

[101] Dr Johnstone provided a report on 22 May 2020 to WorkCover following his examination of the Appellant in relation to the left shoulder claim. In the report Dr Johnstone noted the following –

It is my medical conclusion, as a result of my further review of Mr Peter Richards on 22nd May 2020 and the information sent to me by your Office in respect of his left shoulder and ongoing left elbow complaints, that I can confirm that Mr Richards did not, during my questioning at his Independent Medical Examination on 22nd November 2019, give any information about his left shoulder. I note that, with regards to the Independent Medical Examinations by Dr O'Toole and Dr Scott, the left shoulder was also never mentioned. If, as he says now, that the left shoulder was injured using the sledge hammer to hit the tailgate then he would have been complaining of his left shoulder and left elbow at these appointments. He presented at my latest examination, in respect of his left shoulder, like that to Dr Melsom and he certainly presents with odd behaviour. [emphasis added]

...

Examination of his left shoulder is really consistent with the MRI scan diagnosis of adhesive capsulitis and bursitis.⁹⁷

[102] I find it unlikely that Dr Johnstone would fail to note the Appellant's comments regarding shoulder pain if such comments had been made.

[103] Dr Johnstone gave the following evidence under cross-examination as to whether it was possible that the mechanism of using a crowbar and sledgehammer to open and close the tailgate could exacerbate an underlying degenerative shoulder condition –

It is possible, yes. But, once again, my understanding - that Mr Richards was allegedly complaining that these symptoms started in November 2018; that was over a year - it was about a year since - till I saw him.

..

So if he was going to be made symptomatic by that action in his shoulder then he should have been complaining of symptoms long before he did to his GP.

...

So why - so why hasn't he actually included his shoulder in the gamut of elbow symptoms and ulnar nerve symptoms from the same forceful event? And he didn't - he didn't.

...

It is possible, but I would expect the man to develop symptoms at the time. --- And note any delay in the symptoms. He's developed ulnar nerve symptoms from what is a mild impact on this gear lever. He complained of tennis elbow symptoms when doing the same actions that you were talking about of opening the tailgate on the tip truck in using a crowbar. And if you - by your statement, then he should also be developing shoulder symptoms. So if he's going to complain about the others, then he should've complained about the shoulder being symptomatic as well, if there was of any consequence,

⁹⁷ Exhibit 1, Trial Bundle, Report of Dr Johnstone dated 22 May 2020 [86].

and that has not been what's been reporting, in my understanding.⁹⁸

[104] Dr Johnstone's evidence was that whilst it was possible that the tailgate mechanism described could exacerbate an underlying condition, he would have expected the Appellant to have complained of symptoms.

[105] Dr Johnstone concluded that the Appellant had sustained a 17% upper limb impairment for 'loss of left shoulder motion' based upon his examination of the Appellant on 10 June 2020.

[106] The Appellant submits that the logical conclusion is that a 17% left shoulder related impairment to the same arm as the elbow injury could not entirely be as a result of natural degeneration, contending that it was a result of an employment related aggravation.

[107] In the absence of medical evidence supporting the basis of the Appellant's contention, I cannot be satisfied that the 17% impairment assessment is not a result of a pre-existing degenerative condition given the medical history of the Appellant's shoulder.

Ms Hedi Jen – physiotherapist

[108] The Appellant gave evidence that he told Ms Jen about his left shoulder pain when he commenced treatment for his elbow in March 2020⁹⁹ and that after several appointments, Ms Jen conducted an abduction test on his left shoulder which caused him 'agony'.¹⁰⁰ The Appellant stated that he advised Ms Jen that he had injured his shoulder at the same time that he had injured his elbow,¹⁰¹ and that his left shoulder pain came more to the fore after this abduction test carried out by Ms Jen.¹⁰²

[109] Ms Jen opined that the Appellant had sustained a left shoulder injury from the way he was carrying his arm across his waist guarding the left shoulder and his response to her left arm abduction test.

[110] Ms Jen wrote to WorkCover on 26 June 2020 stating the following –

[The Appellant] has been at a standstill with his rehab for his elbow issues because of his shoulder. Ideally he would benefit from some psychology intervention but I don't think that he is open to seeing anyone else. I have been working slowly to challenge some of his pain and injury beliefs to move forward but the lack of clarity surrounding his initial injury and medical documentation is a big

⁹⁸ T 2 – 23, ll 1 – 39.

⁹⁹ T 1 – 49, l 16.

¹⁰⁰ T 1 – 20, l 1.

¹⁰¹ T 1 – 20, ll 3 – 5.

¹⁰² T 1 – 50, ll 3 – 4.

stumbling block and we are going in circles.¹⁰³

- [111] Ms Jen provided a statement dated 8 September 2022 that was tendered by consent. In the statement Ms Jen notes that the elbow tear and shoulder issue could both be caused by heavy and repetitive work and were consistent with the action that the Appellant described to open and close the faulty tailgate.¹⁰⁴
- [112] The difficulty with Ms Jen's evidence is that she did not examine the Appellant until more than 16 months after the left shoulder injury is alleged to have arisen and the Appellant had ceased employment. In these circumstances, this evidence is of little assistance in determining if the symptoms arose during the relevant period.
- [113] On the balance of probabilities, it is unlikely that multiple medical practitioners neglected to note the Appellant's reporting of shoulder pain. A more plausible explanation for the lack of contemporaneous notes is that the Appellant did not mention shoulder pain as there were no symptoms relating to his shoulder present during the relevant time.
- [114] The note the Respondent's submission that the Appellant is not being truthful about his experiences of pain in his left shoulder and that the claim of a left shoulder injury caused by manipulating the tailgate of his truck between 1 February 2018 and 13 November 2018 is a recent invention. I also note that the reports produced by Dr Melsom¹⁰⁵ and Dr Johnstone¹⁰⁶ mention 'unusual pain behaviour' exhibited by the Appellant upon examination.
- [115] Whilst Dr Melsom states in his report that it is plausible that the Appellant's current symptomatology in his left shoulder arose from work-related tailgate activities, this opinion is given in March 2021, over two years following the period within which the symptoms allegedly arose.
- [116] I also note Dr Melsom's opinion of the 23rd of March 2021 that many of the changes on the Appellant's MRI scan could be ascribed to age related degenerative changes.
- [117] The contemporaneous medical evidence does not support a conclusion that the Appellant reported an injury or pain in his left shoulder during the relevant period. The only reasonable explanation for the Appellant not reporting pain in his left shoulder is that he did not suffer from left shoulder pain during that time. In the absence of reported symptoms during the relevant period when the Appellant was employed or a reasonable

¹⁰³ Exhibit 1, Trial Bundle, Report of Ms Heidi Jen to WorkCover dated 26 June 2020 [206].

¹⁰⁴ Exhibit 3 [1].

¹⁰⁵ Exhibit 1, Trial Bundle, Report of Dr Melsom dated 23 March 2021 [112].

¹⁰⁶ Exhibit 1, Trial Bundle, Report of Dr Johnstone to WorkCover dated 22 May 2020 [86].

time after, there is insufficient evidence to conclude that any injury arose out of or in the course of the Appellant's employment.

[118] An aggravation is established when a previously asymptomatic condition results in pain, as outlined in *WorkCover Queensland v BHP (Qld) Workers Compensation Unit*—

It is settled that the activation of pain is to be equated with the aggravation of an underlying disease; ie. it is sufficient that an asymptomatic disease becomes painful, even if there is no change to the underlying pathology.¹⁰⁷

[119] In the absence of medical evidence of the activation of pain in the Appellant's left shoulder arising during or soon after the relevant period, the Commission cannot be satisfied that the Appellant has sustained an aggravation arising out of or in the course of his employment to which employment was a significant contributing factor.

Conclusion

[120] On the basis that there is insufficient evidence that the Appellant suffered an injury to his shoulder in the form of an aggravation arising of his employment to which employment was a significant contributing factor, I cannot conclude that the Appellant suffered an injury in accordance with s 32 of the WCR Act.

Orders

[121] I make the following orders:

1. **The appeal is dismissed.**
2. **The decision of the Workers' Compensation Regulator is confirmed.**
3. **Failing agreement on costs, to be the subject of a further application to the Commission.**
4. **Liberty to Apply.**

¹⁰⁷ *WorkCover Queensland v BHP (Qld) Workers Compensation Unit* (2002) 170 QGIG 142